

SPECIMEN

- A. Right colon
- B. Apical lymph node

CLINICAL NOTES

PRE-OP DIAGNOSIS: Ascending colon cancer

GROSS DESCRIPTION

A. Received fresh for tissue procurement labeled "right colon" is a 35 cm. segment of proximal right colon with attached 48 cm. of distal ileum surfaced by smooth to scabrous tan-pink serosa with a copious amount of attached mesocolon, mesentery, and unremarkable omentum. An appendix is not identified grossly. The proximal and distal margins measure 3.8 and 6.2 cm. in circumference respectively. The proximal 10 cm. of ileum is adhered to the cecal pouch, which on opening, contains a circumferential 9

x

5.5 cm. rubbery tan-white tumor mass. On sectioning, the tumor has

a

maximal thickness of 3.5 cm., grossly extending through the muscularis to within 0.2 cm. of the inked free radial serosal surface. The mucosa throughout the remainder of the ileum and colonic segment is unremarkable glistening tan-pink with regular folds and the walls average 0.5 cm. in thickness. No additional abnormalities are noted. A portion is taken for tissue procurement as requested. Several soft tan-pink to rubbery tan-white tissues in keeping with lymph nodes measuring up to 2 cm. in greatest dimension are recovered from the attached mesentery and mesocolon. Representative sections are submitted in 16 blocks as labeled. RS-

16

BLOCK SUMMARY: 1 - proximal and distal margins; 2 - tumor with adhered portion of ileum; 3 and 4, 5 and 6 - tumor full thickness

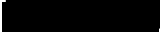
to

inked free radial serosal surface (black) - points of continuity inked orange and green respectively; 7 - tumor to proximal mucosa;

8

- tumor to distal mucosa; 9 - random ileum; 10 - random colon; 11 -

GROSS DESCRIPTION

2 whole lymph nodes; 12 - 6 whole lymph nodes; 13-15 - 1 bisected lymph node per cassette; 16 - 1 representative section from each of 2 grossly positive lymph nodes. 

B. Received fresh labeled "apical lymph node" is a 0.6 x 0.35 x 0.2 cm. fragment of soft to slightly rubbery glistening to chalky golden yellow adipose tissue. A definitive lymphoid tissue is not recovered. The specimen is submitted in toto in one block. AS-1. 



MICROSCOPIC DESCRIPTION

Histologic type: Adenocarcinoma, not otherwise specified.
Histologic grade: Moderately differentiated.
Primary tumor (pT): Tumor invades through the entire colonic wall into the adhered section of ileum into the muscularis propria of the ileum, through the muscularis propria and into the submucosa of the ileum. This is shown in slide A2 (pT4b).
Proximal margin: Negative for tumor.
Distal margin: Negative for tumor.
Circumferential (radial) margin: Not applicable.
Distance of tumor from closest margin: 10 cm from the ileal margin.
Vascular invasion: Prominent lymphatic and venous space invasion is identified.
Regional lymph nodes (pN): 2 of 13 lymph nodes are positive for metastatic carcinoma (pN1).
Non-lymph node pericolic tumor: Focally present in the mesenteric fat between the ileum and colon.
Distant metastasis (pM): Cannot evaluate (pMx).
Other findings: None.

MICROSCOPIC DESCRIPTION

5x1,3260F

DIAGNOSIS

Colon, right, ileocelectomy:
Invasive, moderately differentiated adenocarcinoma, invasive through the entire right colonic wall and through the visceral peritoneum into the wall of the adhered ileum (pT4b).
Metastatic adenocarcinoma present in 2 of 13 lymph nodes (pN1).
The resection margins are negative for carcinoma.
Prominent lymphatic and venous space invasion is identified.

